South London and Maudsley NHS
NHS Foundation Trust

Southwark CMHT - Camberwell & Peckham
1 St. Giles House
St. Giles Road
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SE5 7UD

NB/KS

16th December 2022

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PRIVATE & CONFIDENTIAL
Dr A Oguntimehin
The Acorn & Gaumont House Surgery
153 Peckham High Street
Peckham
London
SE15 5SL

Dear Dr Oguntimehin,

Re: Mr Alfred MAYAKI D.O.B: 18/12/1987 NHS No. 625 082 5304
218 Cator Street, LONDON, Greater London, SE15 5GJ

Mr Mayaki was seen for transfer of care meeting over Teams on 14/12/2022. He remains fairly stable, with mild residual symptoms which potentially contributes to his social anxiety. He is going to be transferred to Southwark LIT (Low Intensity Team), who will continue to issue and administrate Aripiprazole LAI 400 mg once monthly.

I would be grateful if you continue monitoring his physical health, arranging his routine bloods + ECG according to NICE guidance of antipsychotic medication monitoring.

If you have any questions, please do not hesitate and contact us.

Yours sincerely,

Dr Natavan Babayeva (She/her)
Consultant Psychiatrist CMHT Camberwell & Peckham
South London & Maudsley NHS Foundation Trust
St Giles House, St Giles Road, London SE5 7UD
Tel: 020 3228 1800 I Email: natavan.babayeva@slam.nhs.uk

Enc. CPA review notes

ToC meeting via Teams

PRESENT:

Alfred Mayaki
Dr Babayeva, (Consultant)
Tracy W, (CC from LIT team)

DIAGNOSIS:

F25.0 - Schizoaffective disorder, manic type

BACKGROUND

• First known to MH services 2011 while studying for MSc in Economics – disinhibition, grandiosity. Admission S136->S2->S3 to Colchester IP ward. Discharged to STEP on olanzapine 20mg.
• 2012: olanzapine stopped due to side effects and being well for 1yr
• 2013: MH deterioration, admission MHA S2. Discharged on flupenthixol depot. 2014 flupenthixol switched to aripiprazole due to SE.
• Responsive to aripiprazole, in paid employment and dc to GP Feb 2015.
• Re-referral 2015 after deterioration in MH, worked with HTT and dc on Haldol depot.
• 2016: admission to JDW 1/12 under MHA S2. Had stopped AP and disengaged with CMHT. Discharged on aripiprazole depot.
• Has been concordant with depot, at times needs more prompting and reassurance, usually accepts advice from medical professionals

MEDICATIONS:

Aripiprazole 400mg IM monthly last have o 13/12/2022
Previous AP: olanzapine, risperidone, flupenthixol, quetiapine, Haldol decanoate.

PHYSICAL HEALTH:

• Nil PMHx
• Bloods Jan 2019: NAD
• ECG June 2021: sinus tachycardia HR 105, QTc 425ms
• Had both doses of vaccine, reluctant to have a booster

DRUG/ALCOHOL: Hx of cannabis and cocaine use. No current use. Said that he smokes cigarettes.

CURRENT SOCIAL CIRCUMSTANCES:

• Known personal Hx: Born in Nigeria, moved to UK at 4yo
• Accommodation: Lives with mum, in terraced house
• Finances: Working with welfare around UC. Has debt but is struggling to clear them with UC only, managed to pay half as supported by his family. About £500 left, was given some time to clear that by debt collector Applied for PIP
• Social network: supported by mother, maternal aunt/uncle Intermittent contact with father – but not recent. No siblings
• Employment/education. Reported in the past, that he has his own recruitment company, bid for two contracts (public companies), did not get them as he did not have grantor. Said that he meets potential clients and contacts, doing it F2F, could be financially difficult, due to his limited finances, prefers remote contacts via Zoom/Teams. He is not interested in any support from us in terms of his employment.

REVIEW

Seen via Teams
We explained that it is transfer of care meeting
Reports he is doing well, reported good sleep and appetite
Tracy explained LIT structure and frequency of contacts
Discussed his work with welfare and support he had been receiving
Discussed the importance of physical health monitoring, Alfred acknowledged that it is important and agreed to consider it
He was grateful for support he has been receiving from our team so far

MSE

A&B: overweight gentleman, casually dressed, well kempt. Rapport built, good eye contact.
S: spontaneous, normal rate tone and volume, at times rambling
M: objectively and subjectively euthymic, affect reactive. smiling and laughing appropriately
T: no paranoia. tends to over-intellectualise.
P: denies hallucinations in all modalities, not responding
I: accepted his diagnosis, happy to continue with aripiprazole and to engage with CMHT.

RISK

• Risk to health: currently stable, and willing to work with team/take medication
• To self: denies thoughts of self harm/suicide. when unwell self neglects, social isolation.
• To others: denies thoughts to harm others. when unwell can be hostile, sexually inappropriate. Has broken into a house and physically assaulted a woman when acutely psychotic.

IMPRESSION

Schizoaffective disorder, responsive to aripiprazole with mild residual symptoms (mild thought disorder, negative sx which are likely contributing to social anxiety).
On the balance there is not much role for long stay team, apart from giving him a depot, he is stable in the community on Aripiprazole depot since last discharge from hospital in 2016

PLAN

- To be transferred to LIT
- Update risk assessment and care plan
- Ask GP for routine bloods and ECG
- Close to CMHT Camberwell and Peckham
- Tracy will send her contact details and address of Marina House
- Next depot on 13/01/2023 Continue with current treatment